WORK DONE REPORT

FREE PEDIATRIC & CHILD HEALTH CAMPS

Two-Month Community Health Initiative | April–May 2026

Organised as a community-focused health outreach initiative to improve access to pediatric and child health consultation and guidance.



Documentary evidence from the child-health outreach activity.

Reporting Period	April–May 2026
Activity Type	Free Pediatric & Child Health Camps
Primary Beneficiaries	Children and accompanying parents/caregivers
Evidence Available	Photographic documentation from camp/clinical settings

1. Executive Summary

During April and May 2026, a two-month free pediatric and child health outreach initiative was undertaken to provide accessible health consultation and child-focused medical support to families. The activity created a structured setting where children could be brought for clinical assessment and parents/caregivers could interact with healthcare personnel.

The photographic evidence supplied for this report shows: children and families waiting for consultation, a pediatric clinical consultation setting, healthcare personnel attending to children, and a camp environment displaying programme information associated with Bhat Foundation Welfare Society and Ibn Sina Hospital.

At-a-Glance

Period	Focus	Beneficiaries	Delivery Model
April–May 2026	Pediatric & child health	Children + caregivers	Free community health camps

2. Purpose of the Initiative

- Improve access to basic pediatric health consultation for families who may face financial or access barriers.
- Create an approachable community setting for parents and caregivers to seek guidance regarding children's health concerns.
- Facilitate child-focused clinical interaction through organised camp-based services.
- Promote early attention to health concerns by connecting families with healthcare professionals.
- Document community-level health outreach for accountability, reporting and future programme planning.

3. Target Beneficiaries

The initiative primarily served children and their parents/caregivers. The available photographs show a mixed group of families, including children accompanied by mothers, fathers and other caregivers. The supplied evidence does not provide a verified numerical beneficiary count, so no beneficiary total has been inserted into this report.

4. Programme Implementation

The two-month activity was implemented through free pediatric and child-health camp sessions. The available photographs document the patient flow and clinical environment, including waiting areas, consultation rooms and direct interaction between healthcare personnel and patients.

Implementation Component	What the Evidence Shows
Community access	Families were received in a dedicated healthcare environment and guided through the camp process.
Waiting & registration flow	Photographs document groups of children and adults waiting for consultation and carrying documents/forms.
Clinical consultation	A pediatric/clinical consultation setting is visibly documented, with healthcare personnel examining a child.
Parent/caregiver interaction	Caregivers accompanied children during consultation and remained involved in the care process.
Health guidance	The clinical setting supports consultation and communication between healthcare personnel and families.

5. Key Activities Conducted

- Free pediatric and child health consultation sessions conducted during April and May 2026.
- Child-focused clinical assessment and examination within a healthcare facility setting.
- Parent/caregiver participation during the consultation process.
- Organised waiting and patient-flow arrangements visible in the camp documentation.
- Health outreach delivered without consultation charges to the participating beneficiaries, as described by the programme title supplied for this report.

6. Service Delivery Environment

The photographic record demonstrates a functioning clinical environment with consultation rooms, waiting areas, healthcare personnel and families. The images also show a programme banner identifying a free medical camp associated with Bhat Foundation Welfare Society and Ibn Sina Hospital. Specific clinical procedures, investigations, medicines or referrals are not claimed here unless independently documented.

7. Community Impact

The principal impact of the initiative was the creation of a free, organised access point for pediatric and child health consultation over a sustained two-month period. The photographs provide direct documentary evidence of participation and service delivery.

Impact Area	Observed / Supported Outcome
Improved access	Free camp-based consultation reduced the immediate financial barrier associated with accessing pediatric consultation.
Child-focused care	The activity created a dedicated environment for children to receive clinical attention.
Family engagement	Parents and caregivers were present throughout the patient journey, supporting informed participation in children's care.
Community outreach	The camp model brought healthcare services into a community-facing programme rather than relying only on routine hospital visits.
Documented delivery	Photographs provide visual evidence of beneficiaries, waiting areas, consultation and programme signage.

8. Outcome Framework

Level	Expected Change	Evidence in Record	Status
Input	Healthcare personnel, facility and outreach coordination	Clinical setting visible	Implemented
Activity	Free child health consultation	Consultation photographs	Implemented
Output	Children/families reached through camps	Waiting and consultation images	Documented
Immediate outcome	Improved access to pediatric consultation	Families receiving services	Supported by photos
Longer-term outcome	Earlier attention to child health needs	Not measurable from supplied evidence	To be tracked

9. Photographic Documentation

The following photographs are included as primary documentary evidence of the activity. Captions describe only what is visibly supported by the supplied images.



Photo 1 — Children and caregivers waiting for consultation in the healthcare facility.



Photo 2 — Pediatric/child consultation setting with healthcare personnel and accompanying family members.

10. Additional Documentary Evidence



Photo 3 — Clinical examination of a child during the documented health outreach activity.

11. Accountability & Documentation Note

This work-done report has been prepared from the programme description supplied by the organisation and the photographic evidence provided for the April–May 2026 activity. Where numerical figures such as total beneficiaries, number of consultations, investigations, medicines distributed or referrals are not present in the supplied evidence, they have intentionally not been estimated.

12. Recommended Monitoring Indicators for Future Camps

- Total children registered and total children clinically consulted.
- Age-wise and gender-wise beneficiary distribution.
- Number of first-time versus follow-up consultations.
- Common health concerns identified during consultations.
- Number of investigations/tests conducted, if applicable.
- Number of medicines/support items distributed, if applicable.
- Number of referrals made for further treatment.
- Beneficiary satisfaction and parent/caregiver feedback.
- Geographical coverage and number of participating communities.

13. Conclusion

The April–May 2026 Free Pediatric and Child Health Camp initiative represents a sustained two-month effort to connect children and families with accessible healthcare consultation. The available photographs document active participation, organised patient flow and direct clinical interaction. The initiative can be strengthened in future cycles through systematic beneficiary registration, service statistics, referral tracking and follow-up documentation, which would allow the organisation to quantify health outcomes alongside the existing photographic evidence.

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